

AMT-130 (uniQure, NASDAQ: QURE): investment memo

Daniel Haselbach · 1 October 2026

Frozen at the 1 October 2026 close: \$23.37 (Nasdaq). Balance sheet 30 June 2026 (Q2 10-Q), cash rolled to an estimated 30 September. Four-year data (29 September 2026) included. Per-share figures fully diluted, options treated as exercised at fair value.

1. The call

Buy QURE at \$23.37 (1 October 2026 close): after tax and development costs, AMT-130 alone is worth \$30.04 per fully diluted share, 29% upside. The upside comes from approval: my cash flows are worth the price at a 40% probability of success (PoS), and I put it at 58% (section 2). Whether that upside is paid depends on surgical capacity. Each dose is an 8–10 hour neurosurgery, and most of the centres my base case needs have not yet performed it. If they reach my base case, the FDA's accelerated-approval decision, which I model for mid-2027, closes the gap; if they stall, no defensible PoS supports even today's price (section 4). uniQure's launch-strategy update, due early 2027, tests the capacity case before the FDA decides. The upside does not survive a slip to 2029 (\$23.50) or treatment of only half the eligible pool (\$23.94).

2. Probability of success: 58%

58% is my probability that AMT-130 is approved and keeps that approval through the confirmatory study. The base case times approval for mid-2027; a slip to 2029 is a separate downside case (section 4).

Anchor: 86.7%, the NDA/BLA-to-approval success rate for neurology over 2011–2020 (n=165; Wilson 95% interval 80.6–91.0%, my calculation) [1]. Not the ~46% Phase III-to-approval rate: that multiplies in a 53.1% chance of reaching a filing [1], and uniQure has filed.

–30: the benefit is measured against a control the sponsor built, and the control has moved. The BLA rests on 12 high-dose patients at 36 months compared with 940 propensity-matched patients from the Enroll-HD natural-history study [2]. The four-year analysis uses a different comparator, a newer Enroll-HD release in which 53% of the matched controls' 48-month data are missing; against it, the cUHDRS effect fell to 44% and lost significance (p=0.144) [3]. uniQure now argues that the controls who dropped out were declining faster, so its own control understates progression [3]. That may be right, and it is the problem: rebuild the comparator and the effect moves with it. No multiplicity procedure is disclosed across the five endpoints; at 48 months the company labels every p-value except the primary as nominal [3]; at 36 months SDMT (p=0.057) and TMS (p=0.1741) missed [4]. Propensity matching assumes matched patients progress alike, yet patients with PIN prognostic scores near zero go on to enlarge their ventricles by 100 to 3,000 mm³ a year [5]. None of this says the effect is small: a 1.14-point cUHDRS gap at 36 months (–0.38 vs –1.52) is not small if it is real [4]. It says this comparison cannot show that it is.

+3: the June position, net of how often the FDA has moved. In March 2026 the FDA said the external-control data could not be the primary evidence and "strongly recommended" a randomized, double-blind, sham-surgery-controlled study [2]. In June, per uniQure's 8-K, it said the three-year analysis "would be acceptable as the primary basis of a BLA for the accelerated approval of AMT-130" [8]. The official minutes, received in July, read weaker in uniQure's own account: they confirmed alignment that a BLA submission "is reasonable" [11]. That gap does not move the number. Two months

later, with the minutes in hand, uniQure still wrote "acceptable as the primary basis" [3], and the credit needs only the weaker wording: it is for what the FDA will review, not what it will conclude. June is worth +8. I take 5 back because it was the agency's fourth position in nineteen months, after agreeing in November 2024 [6], withdrawing that agreement in November 2025 [7] and the March recommendation, from an agency that, in a senior official's words, "as a general rule, never makes such assurances" [9]. Filing itself earns nothing: the anchor already starts from filed applications.

–5: advisory committee. Management "most likely" expects one [10]. The deduction is for the process: a public vote on contested evidence, by members who were not in the June meeting. Before the June reversal, a senior FDA official, speaking anonymously, called AMT-130 a "failed product" [9]. That is evidence of what the review division brings into the room, not another position change.

–2: confirmatory timing. uniQure says the FDA wants the confirmatory study "well underway, and potentially fully enrolled, at the time of accelerated approval" [11]; screening is targeted to start only before the end of 2026 [12].

+5: unmet need. No available therapy delays onset or slows progression of Huntington's disease [2]. $86.7 - 30 + 3 - 5 - 2 + 5$, rounded: 58%. The anchor is a measured rate; the adjustments are my judgement.

The confirmatory study sits inside the –30, not in a separate deduction. The 58% includes keeping the approval. The –30 is my doubt that the effect is real; a randomized study against standard of care removes the comparator problem and answers that same question later, so I do not charge for it twice. The argument has one weak point: the confirmatory primary endpoint, total functional capacity at 36 months [10], held at 61% slowing at four years (nominal $p=0.008$) [3], but against the same external control. A fading effect threatens the approval and invites outcomes-based payer contracts; it also sits inside the 58%, not in a separate revenue haircut.

Where I sit. At \$23.37 my cash flows are worth the price at a 40% PoS. That is not the market's probability but the one that reconciles my model with the price, and an upper bound on it, because the rest of the pipeline gets zero. Leerink carried 75% for the US in June, on larger cash flows than mine [13]; after the readout, Stifel called the three-year cut "the crux of QURE's alignment with FDA" [14]. I sit at 58%: below the sell side because the comparator is fragile, above 40% because uniQure repeated the June wording after the official minutes, filed its BLA on it, and the FDA has not publicly contradicted it. The June position reaches me only through uniQure's account, which is why I stop at 58%.

3. rNPV \$1,528M; equity value \$30.04 a share

Method. Revenue is patients treated times price, limited each year by surgical capacity and in total by the eligible pool. Contribution is 51% of revenue, my assumption, set at the margin Sarepta earned on 2024 product sales after cost of sales and SG&A [15]. A 5% Cold Spring Harbor royalty runs to 2031; the 10-K says only "single-digit" and that the licensed patents expire in 2031 [2], so both are my assumptions. AMT-130 R&D is held at its FY2025 level of \$42.5M a year [2] from Q4 2026 through 2031, my assumption, and profit after R&D is taxed at 21% [2]. Cash flows are discounted to 1 October 2026 at 8.49%, Damodaran's January 2026 cost of capital for biotechnology [16], and the contribution is risked at 58% (R&D is not): an rNPV of \$1,528M.

To reach equity I add cash rolled to an estimated \$766.9M at 30 September (\$810.3M reported at 30 June [11], less a quarter of underlying burn), subtract the \$50M Hercules loan [2], and treat every option as exercised at fair value. The royalty-financing liability is excluded: uniQure has no obligation to repay it beyond the lowest HEMGENIX royalty tier it sold [2], which I value at zero, as I do AMT-260, AMT-191 and the retained HEMGENIX economics. From the pre-tax engine: $\$36.91 + \1.68 (discounted to 1 October, not 1 January) – $\$1.89$ (R&D) – $\$5.77$ (tax) – $\$0.58$ (cash rolled from 30 June to 30 September) – $\$0.31$

(dilution) = \$30.04.

Patients and price. 40,000 symptomatic patients in the US [2] and about 8,400 in the UK (12.3 per 100,000 [17] in a population of 68.3 million [18]) make 48,401. Of these, 23.4% are early-manifest, so 11,326 are eligible. The 23.4% is the early-stage share of Medicare (65+) prevalent cases [19]; applying it to all US and UK symptomatic patients is my extrapolation. It holds across ages: the same paper gives 22.6% for Medicaid patients under 65 [19]. Every pool percentage below uses the 11,326 as its denominator. I assume 75% are treated by 2037: 8,494 patients. That percentage is my assumption, and I have no source for it. My reason is that these patients have no therapy that slows the disease [2], so the alternative to surgery is no treatment at all. Apart from capacity, it is the input the call is most exposed to: at 60% fair value is \$26.49, at 50% \$23.94, and below roughly half the eligible pool it falls under the price. Leerink gives \$1.7M for the US and \$1.0M for the EU without saying whether these are net, list or gross [13]; treating them as net is my assumption, and the UK takes Leerink's EU figure as a proxy.

Capacity: this memo's own contribution. Each administration is an MRI-guided neurosurgical infusion into six sites deep in the brain, taking 8–10 hours [20]. uniQure calls it "a standard procedure for a neurosurgeon", counts "approximately 50 to 55 sites with the necessary neurosurgical expertise and imaging equipment" in answer to a question about US sites, and plans "a center of excellence strategy" at launch [21]. The constraint is not skill but suite time. Today's footprint is small: ClinicalTrials.gov lists 12 US sites for the Phase I/II study, one a non-surgical research site, so at most 11 have performed the procedure [22]; in the UK, Cardiff is the only surgical site [23]. My base case grows from 8 centres doing 6 cases a year in 2027 to 55 US and UK centres doing 25 by 2034: 1,375 patients and \$2,170M a year. Fifty-five is in line with the CEO's US figure, not conservative against it. Twenty-five cases a centre is unproven for this procedure, not impossible: uniQure finds it "very challenging to give you one number" [10], and high-volume US deep-brain-stimulation centres run 75 or more cases a year [24]. Twenty-five cases is 200–250 hours of suite time per centre a year. That number is mine.

The ramp from 8 to 55 centres and from 6 to 25 cases is where ecosystem growth already sits, and uniQure says it is "advancing our commercial preparations" [12]. A faster buildout is upside, in section 5, not counted here. The confirmatory study's treated arm needs the same surgery at the same centres; I give it none of these slots, which is generous to uniQure [10].

Leerink's \$2.5B peak is worldwide gross [13], a ceiling on my US+UK model that never binds at Base or Low and clips High in 2032–34. Even if the US and UK carried all of it, 55 centres would need about 29 cases each a year: an upper bound, not an implied throughput.

Capacity changes timing, not patient count, until it doesn't. At High (55 centres × 40 cases) the same 8,494 patients are treated sooner (section 5). At Low (40 × 12) capacity peaks at 480 patients a year, the pool is never exhausted before exclusivity ends, cumulative revenue is \$6,336M against \$13,408M at Base, and \$12.10 a share is lost (section 4).

Exclusivity. Revenue ends in 2037, when the 75% has been treated, two years before exclusivity ends in 2039. That date rests on the 12-year exclusivity the BPCIA gives a biologic first licensed in 2027: statutory, but subject to the FDA's first-licensure determination, and uniQure says it intends to apply for all periods of exclusivity it may be eligible for [2]. A four-year extension of US patent 10,174,321, which expires on 23 December 2035 [25], reaches the same date; the four is my assumption. Without either, protection ends with 2035: cumulative revenue falls to \$11,166M and fair value by \$2.93 (counting 2035, which the engine's convention drops).

Not in the number: ex-US named-patient and early-access programmes, which a US or UK approval "would also unlock", per uniQure [10]; and Dutch tax losses, worth about \$0.72 a share only if profit is taxed at the Dutch 25.8%, where fair value is \$28.78. The pool is a fixed stock: it leaves out patients who reach the early-manifest stage after today and today's patients who progress past it before a slot opens. I assume the two offset.

4. What would make me wrong

An advisory committee weighs the four-year data over the three-year package: 44% cUHDRS slowing, not significant ($p=0.144$) [3]. The application fails.

Centres stall. At Low capacity fair value is \$17.94, and \$23.37 needs an 89.8% PoS, above the 86.7% base rate for filed neurology applications [1]. No defensible PoS supports the price.

The confirmatory study fails. Screening starts by end-2026 and the primary analysis is at three years [12]: a failure, and the "expedited withdrawal" it can bring [2], lands after launch, with 84% of my present value (2031–37) still ahead.

Price lands below \$1.7M, or payers claw back on a fading effect. Net price is my assumption [13]. Fair value reaches \$23.37 only at 69.3% of Leerink's prices (\$1.18M US, \$0.69M UK), a 31% cut.

Protection ends with 2035: −\$2.93, to \$27.11.

A rival takes patients. The 12-year exclusivity blocks only biosimilars [2]. Votoplam, a "small molecule splicing modifier", is in Phase 3 INVEST-HD: 770 participants, 3:2 against placebo, cUHDRS through month 36 as primary [26]; started 24 March 2026, completion listed for 30 April 2030 [27]. It requires a TFC of exactly 13, the top of a 0–13 scale [27, 28], so the trial overlaps only my pool's earliest slice; as "a small molecule that can be taken orally" [29], it competes for patients, not surgical suites. By end-2030 my base case has treated 1,004 patients, 12% of its total. At 50% of the eligible pool treated, not 75%, fair value is \$23.94, 2% above the price.

Approval slips to 2029. Every AMT-130 cash flow moves two years and the company burns two more years of cash: −\$6.54, to \$23.50, 13 cents above the price. The asset survives; the trade doesn't.

5. What would change my mind

Centre and throughput disclosure. uniQure plans to give "more color on launch strategy early next year" [12]. It is the first test of the capacity case, and it comes before the FDA's decision. Centre counts or cases per centre above my ramp move me toward High; below it, toward Low and section 4.

Faster buildout: +\$1.61. At High, Base centres reaching 40 cases a year by 2034, fair value is \$31.65. The gain is small because the pool caps revenue: more capacity treats the same 8,494 patients sooner.

A learning curve. If experienced centres cut the 8–10 hour procedure [20], cases per centre rise without new suites.

A bid. A buyer would also get AMT-260 and AMT-191, which I value at zero, so a bid would put a floor under the shares. It could also cap the upside below \$30.04; the call assumes neither.

Watch list, in date order. Mid-November: the FDA's day-74 letter on the BLA uniQure announced on 2 September [30], which will include the agency's preliminary plans on whether to hold an advisory committee [31]; uniQure has requested priority review, which, if granted, would bring the decision six months after the 60-day filing period [30]. Early 2027: the launch-strategy update. Then the advisory committee, if the FDA holds one, ahead of the mid-2027 decision.

This memo is the author's independent analysis, prepared for educational purposes. It is not investment advice or a recommendation to buy or sell any security. The author holds no position in uniQure N.V. (NASDAQ: QURE) and received no compensation for this work. Information is as of October 1st and may have changed.

Sources

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